

Tops Tips on Managing CFS/ME Claims

What you can do:

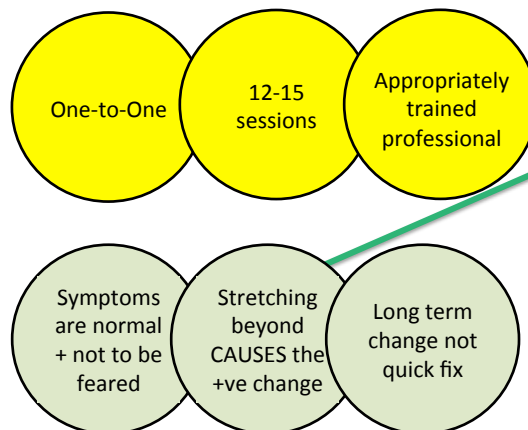
- ☑ Believe and validate: this is a genuine illness diagnosed by doctors
- ☑ Ensure claimants have had optimal treatment, validated by specialists
- ☑ Ensure claimants have had a full biopsychosocial assessment
- ☑ Ensure claimants have had the right diagnosis
- ☑ Ensure sleep and mood has been adequately addressed
- ☑ Refer to specialist CFS/ME teams where diagnosis or treatment is in doubt: but ONLY if you are sure they are offering optimal treatment
- ☑ Intervene early: keep people at work where possible + get help quickly
- ☑ Be prepared to fund treatment where not available on the NHS
- ☑ Be patient with recovery and RTW timescales
- ☑ Be positive about rehabilitation + recovery

Act NOW!
Time =
Money

Effective therapies:

Supported by NICE guidelines + recent research

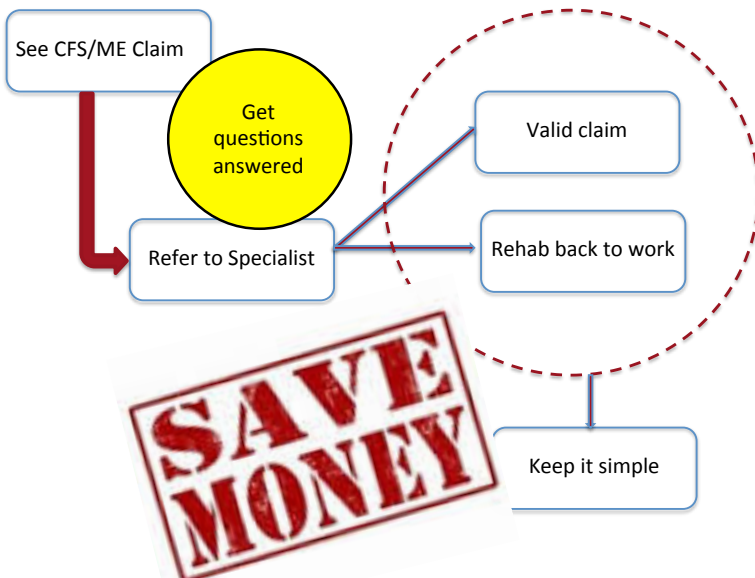
Facilitate +ve
diagnosis and
effective
therapy



Deliberately structured, goal-focussed + incremental:

Graded Exercise Therapy:
Gradual, increases physical activity BEYOND current ability *by planning specific exercise*

Cognitive Behavioural Therapy:
Gradual increase activity BEYOND current ability *by addressing fears + beliefs*



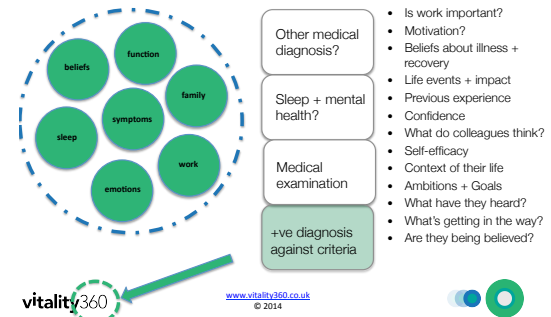
Find a
specialist
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trust

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Assessment + Diagnosis:

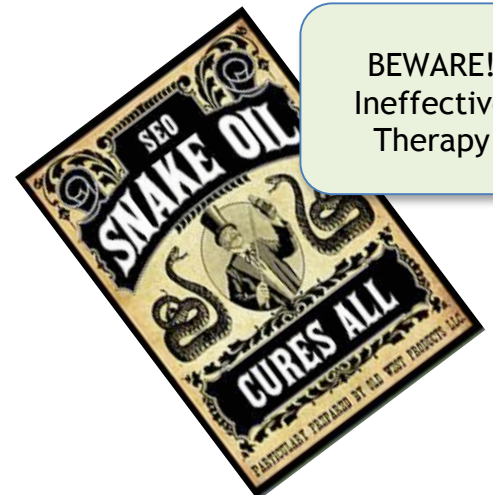
	Question	+ve	-ve
✓	Thorough biopsychosocial assessment?	Takes more than 8 mins! 1-2 hours subjective examination likely	Didn't consider the full picture, or assessed very quickly or unidimensional
✓	All medical tests as per NICE guidelines (p140+)	Can prove tests have been done + other conditions ruled out	Missing or inconsistent tests, unidimensional testing
✓	Psychiatry (where approp) : to rule out mental health conditions	Liaison psychiatry helped inform the diagnosis, or no sign of MH issue	Emotional health + diagnosis not considered in presence of MH issue
✓	Has sleep apnoea been ruled out, especially in overweight, snoring, males with fatigue?	Specifically assessed and ruled out (sleep tests)	No mention of the possibility in susceptible groups – ie not ruled out
✓	Positive diagnosis	Meets specific positive diagnostic criteria (NICE p145+)	<i>"Well, if there's nothing else wrong it must be CFS/ME"</i>

360 BPS Assessment:



Treatment:

	Question	+ve	-ve
✓	At least 12 sessions of GET or CBT for CFS/ME, delivered 1-2-1?	Therapy provider can prove they have done evidence-based treatment (NICE p188+)	Group, brief, under 12 sessions, lacks clarity in approach or message, not person-centred
✓	Therapy message to challenge <i>beyond</i> current capacity? (incremental)	Yes: going beyond is safe, + is what will help get you better	No: Stay within your limits + going beyond could be harmful
✓	Is the therapist appropriately trained?	Therapist can prove training in GET or CBT treatment protocols for CFS/ME specifically	No evidence or insufficient training or supervision, generic CBT or 'exercise'
✓	Has treatment been compared to best practice by specialist?	Specialist has explored treatment efficacy against evidence-base	Assumption made about treatment efficacy without exploring further
✓	Any additional sleep or mental health issue being treated?	Sleep or MH issue separately treated as well as their CFS/ME	Sleep or MH issue ignored or inadequately treated



Vocational Management:

	Question	+ve	-ve
✓	Is claimant going back to work too soon?	Specialist has advised on timing + baseline of work, structure + role	Pressure to go back too soon, ill considered plan, baseline too high
✓	Have all CFS/ME adjustments been considered?	Include work from home, travel, environment, GRTW, support, rehab	No: Specific CFS/ME adjustments or rehabilitation not been considered
✓	Is the rate of RTW to high? ie steps being made too quickly?	Body adapts to each stage before moving onto next level	Next level instigated before adaptation has occurred
✓	Has RTW plan been devised with CFS/ME specialist + employer?	Agreement negotiated between all parties to form workable plan	Lack of specialist support or employer input, no or poor plan in place
✓	Is the goal to RTW being proactively managed by therapist?	Vocational issues central to the rehabilitation plan and treatment	Vocational issues not addressed or managed, no focus or attention

Adjustments for CFS/ME:

- Physical tasks
- Sitting
- Standing
- Concentration
- Computer use
- Multi-tasking
- Noise & multisensory
- Journey to work
- Rest breaks
- Intensity: management, problem-solving, making important decisions
- Ability to switch between tasks
- Interpersonal issues
- Social engagement

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